

This is for someone who wants the company in one sitting.

The stake

eCHIS does not order which sick child goes first. The care path can then fail at the decision, destination, facility, travel, acknowledgement, or returned-result step. Tunza has to exist as an attempt to solve that gap. The demo does not do this yet.

Whether that wait kills a child is unknown; no checked source documents that death. What is documented is the country's toll: [the WHO Kenya 2025 report](#) puts maternal mortality at an estimated 355 deaths per 100,000 live births, from preventable causes, and Kenya's Ministry of Health ties a fragmented referral system to maternal and child health outcomes in the referral policy it is developing. Those are the country's numbers, not Tunza's results.

The Ministry of Health is developing a national referral policy on continuity, interoperability, and accountability. The Ministry ties that work to maternal and child health outcomes. That is public context in a discussion brief. It is not evidence that Tunza implements the policy, and it is not a partnership. Reference only, not an endorsement: <https://health.go.ke/kenya-moves-strengthen-patient-referral-system-through-new-national-policy>

Who it is for

A household. A community health worker, the CHP in this demo. The facility that receives the person.

A household can start with no worker grant. CHP and facility sit behind a placeholder gate that runs in demo mode until real identity exists.

What a person can do in this demo

The words on screen are English and Kiswahili. A person can check symptoms, look at nearby facilities, or continue what is already on that device.

The check takes a short account in text. Voice is one tap to speak, then type if speech is not available. A photo can be marked attached. The rules do not look at the picture.

The result is one of four next steps: go now, get care today, monitor at home, or need one more answer.

What it is not

It is not a doctor. It is not a diagnosis. The rules are a demonstration. They are not clinically validated. The screen says this is not a substitute for emergency services or professional medical care.

The case stays on that device. It is not a shared record. The referral uses a fake demo facility, not a live place that has confirmed it can take the person.

Where it is trying to go

Intention only. We want a referral followed far enough to learn whether the next step was right. That is designed. It is not built. Ordering cases and this question-and-next-step demo are not the same shipped thing.

Sources

- Queue: fact-sheet-urgent.md S1. Six failures: S2–S7. Death link: U1, unknown.
- "Has to exist" is intent, not a measured effect. Demo limit: the honest product boundary in fact-sheet-urgent.md.
- Policy context, not a partnership: the Kenya policy section in fact-sheet-urgent.md.
- Roles and demo: ../tunza-vision-2026-09-23/fact-sheet.md lines 22–39. Intention: line 57.